



3000839695

Ms. PRANATI RAMAKRISHNA BHAT50, HOSTOTA, BELAKANDA,
YELLAPUR, NANDOLLI,UTTARA
KANNADA

Tel No : 9483109545

PIN No: 581359

UHID : MR550803 \ 6D

Age:13Y/1M/11Days / Female

Reference: Dr. Shiva Shankar Bhat

Order No: 3000839695

Sample Collected At:
A J HOSPITAL & RESEARCH
CENTRE MANGALORE.
575004Registered On:
25/07/2025 01:09:00 PMCollected On:
25/07/2025 01:37:45 PMReported On:
25/07/2025 03:48:58 PM

Ph.No: 8494890444

Test Name**REPORT STATUS: FINAL****Result****Unit****Biological Reference****Interpretation:**

< 10 : Deficient

10- 30 : Insufficient

30 - 60 : Sufficient

> 100 : Toxic

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

2. Testing for 25(OH)vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure & dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.

3. During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 mths of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal(especially winter) & individual variability depending on age, body fat, sun exposure, physical activity, genetic factors(especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism Vitamin D toxicity is known but very rare.kindly correlate clinically, repeat with fresh sample if indicated.

Associated Test Profile :

- For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D and serum PTH.An inverse relationship exists between PTH and 25(OH)D levels, Parathyroid hormone levels start to rise at 25(OH)D levels below 31 ng/mL & usually decrease after the correction of vitamin D insufficiency.Thus, restoration of PTH and 25(OH)D levels to normalcy after adequate vitamin D replacement therapy is a useful monitoring strategy.
- As a holistic & scientific approach for diagnosis and optimal treatment for vitamin D deficiency, Vitamin D plus profile (25 Hydroxy(OH) Vit D and PTH) is suggested.

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& Research
Centre**



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26/07/2025 03:48:58 PM**Test Name****REPORT STATUS: FINAL****IMMUNOASSAY****TSH - serum**

(Chemiluminescent Microparticle Immunoassay)

Result**Unit****Biological Reference**

1.034

μIU/mL

0.7-5.3

Interpretation:**Interpretation TSH :**

1. TSH results between 4.5 to 15 show considerable physiologic & seasonal variation, suggest clinical correlation or repeat testing with fresh sample .
2. TSH results between 0.1 to 0.45 require correlation with patient age & clinical symptoms.
As with increasing age, there are marked changes in thyroid hormone production, metabolism & its actions resulting in an increased prevalence of subclinical thyroid disease.
3. TSH values may be transiently altered because of non thyroidal illness like severe infections,liver disease, renal and heart failure,severe burns, trauma and surgery etc .
4. Drugs that decrease TSH values e.g:L-dopa,Glucocorticoid
5. Drugs that increase TSH values e.g iodine,Lithium,Amiodarone.

References:

1. Interpretation of thyroid function tests. Dayan et al. THE LANCET • Vol 357 • February 24, 2001
2. Laboratory Evaluation of Thyroid Function, Indian Thyroid Guidelines, JAPI, January 2011,vol. 59

Report issued is for sample received

25 Hydroxy Cholecalciferol(VitaminD3)**23.1**

ng/mL

30-100

(CMIA)

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(Infrared detection based on Modified Westergren's)

Result**39****Unit**

mm/hr

Biological Reference

1-7

Interpretation:

Principle: Automated ESR measures the erythrocytes kinetics aggregation in agglomerin presence.

Red cell aggregation by kinetic capillary photometry .

Note: Capillary photometry technology overcomes the variables and limitations of sedimentation method.

Interpretation:

1. It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
2. It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
3. It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism



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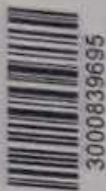
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	Result	Unit	Biological Reference
Lymphocytes (Fluorescence flow cytometry)	16.9	%	20-40
Monocytes (Fluorescence flow cytometry)	9.4	%	2-10
Eosinophils (Fluorescence flow cytometry)	0.1	%	1-6
Basophils (Fluorescence flow cytometry)	0.1	%	0-1
Blood - Platelet count (Flow cytometry)	320	10 ³ / μ l	150-450
Nucleated RBC (Electrical impedance with hydrodynamic focusing)	0.0	NRBC'S / 100 WBC'S	
MPV (Mean Platelet Volume) (Flow cytometry)	8.6	fl	6-9.5

Interpretation:

EDTA Whole Blood - Tests done on Automated Six Part Cell Counter. (WBC, RBC Platelet count by impedance method flow cytometry, WBC differential by VCS technology other parameters calculated) All Abnormal Haemograms are reviewed confirmed microscopically. Differential count is based on approximately 10,000 cells



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REPORT STATUS: FINAL

Test Name

Result

Unit

Biological Reference

HEMATOLOGY

#Blood - Routine (CBC)

Blood - Haemoglobin (Cyanide-free SLS haemoglobin)	14.0	g/dl	11.5-15.5
Erythrocyte (RBC) Count (Electrical impedance with hydrodynamic focusing)	4.65	million/cu.mm	4-5.2
PCV (Packed Cell Volume) (Flow cytometry)	39.1	%	35-45
MCV Blood (Calculated)	84.1	fl	72-95
MCH (Mean Corpuscular Hb) (Calculated)	30.1	pg	25-33
MCHC (Mean Corpuscular Hb Concn.) (Calculated)	35.8	%	31-40
RDW (Red Cell Distribution Width) (Calculated)	12.4	%	11.5-14
Blood - Total Count (TC) (Fluorescence flow cytometry)	<u>15.35</u>	10 ³ / μ l	5-13
Absolute Neutrophils Count (Flow cytometry)	<u>11.27</u>	10 ³ / μ l	2-7
Absolute Lymphocyte Count (Flow cytometry)	2.59	10 ³ /uL	1-3
Absolute Monocyte Count (Flow cytometry)	<u>1.45</u>	10 ³ /uL	0.2-1
Absolute Eosinophils Count (Flow cytometry)	<u>0.02</u>	10 ³ /uL	0.04-0.45
Absolute Basophils Count (Flow cytometry)	0.02	10 ³ / μ l	0.02-0.1
Neutrophils (Fluorescence flow cytometry)	<u>73.5</u>	%	40-70

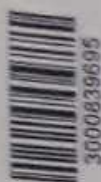
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Interpretation:

Interpretation & Remark:

- HbA1c is used for monitoring diabetic control. It reflects the estimated average glucose (eAG).
- HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association). As recommended by ADA, patients in the range of 5.7% to 6.4% HbA1c would be in the category of increased risk for diabetes and results $\geq 6.5\%$ may aid in the diagnosis of Diabetes.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- Low glycated haemoglobin (below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
- To estimate the eAG from the HbA1C value, the following equation is used: $eAG(mg/dl) = 28.7 \times A1c - 46.7$
- Interference of Haemoglobinopathies in HbA1c estimation.
 - For HbF $> 25\%$, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
 - Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status.
 - Heterozygous state detected (D10/ Tosho G8 is corrected for HbS and HbC trait).
- In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control.
 - Excellent Control - 6 to 7 %,
 - Fair to Good Control - 7 to 8 %,
 - Unsatisfactory Control - 8 to 10 %
 - and Poor Control - More than 10 % .

Note : Hemoglobin electrophoresis (HPLC method) is recommended for detecting hemoglobinopathy.
Report issued is for sample received

#Liver Function Test- Serum

Bilirubin Total, Direct, Indirect Serum	0.43	mg/dL	0.3-1.2
Bilirubin Total Serum			
(Colorimetric Diazo Method)	0.14	mg/dL	0-0.5
Bilirubin Direct Serum			
(Diazo Method)	0.29	mg/dL	0-2
Bilirubin- Indirect, serum			
(Calculated)			

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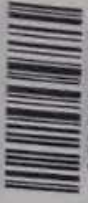
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BIOCHEMISTRY

#Blood Urea- Serum

BUN Urea Nitrogen Serum (Urease with GLDH)	9	mg/dL	6-20
UreaSerum (Enzymatic Colorimetric Method)	<u>19.26</u>	mg/dL	20-40

Interpretation:

In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14.
Report issued is for sample received

#Glycosylated Haemoglobin (HbA1C)

HbA1C- Glycated Haemoglobin, blood (Colorimetry - N-terminal fructosyl dipeptides (Protease +fructosyl dipeptides oxidase))	<u>5.2</u>	%	
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PONNIMA DIAGNOSTIC CENTRE

(Computerised Blood Testing Centre)



D.T. Road, Yellapur (U.K.) Contact : 08419-261335, M : 9019177272

Patient's Name : PRANATI

14 Years

Ref. By. : Dr V.S.PANDIT MBBS

Date: 25/07/2025

Test	Result	Normal Range	Unit
Serum Amylase	: 990	Upto 80	U/L
E.S.R (Westergren's method)	: 81	0 - 15	mm/1st h
C.R.P METHOD: (Turbi Latex Method)	: 56.5	00 - 6.00	mg/L

LIVER FUNCTION TEST

Test	Result	Normal Range	Unit
Bilirubin, Total	: 0.6	0.1 - 1.2	mg/dl
Bilirubin, conjugated	: 0.1	0.0 - 0.3	mg/dl
Bilirubin, unconjugated	: 0.5	0.1 - 0.9	mg/dl
S.G.O.T	: 13.8	0 - 40	IU/L
S.G.P.T	: 20.9	0 - 40	IU/L
Alkaline Phosphatase	: 140	25 - 145	IU/L
Total Proteins	: 6.9	6.0 - 8.0	gm/dl
Albumin	: 3.6	3.5 - 5.5	gm/dl
Globulins	: 3.3	0 - 2.5	gm/dl
A/G Ratio	: 1.09		

Dr. V.S. Pandit
Ponnama Diagnostic Centre
Yellapur (U.K.)

POORNIMA DIAGNOSTICS CENTRE YALLAPUR

First Name: PRANATI

Last Name:

Gender: Female

Age: 14 Year

Sample Type:

Department:

Med Rec No:

Sample ID: 11269

Test Time: 25/07/2025 14:41

Diagnosis:

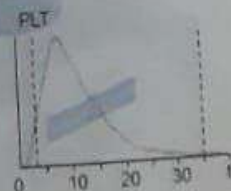
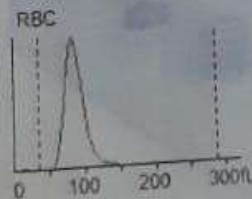
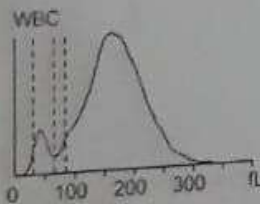
Ph: 08438-251235
Call: 95151-71272

Parameter	Result	Ref. Range	Unit
1 WBC	17.03 ↑	4.00-10.00	10 ⁹ /L
2 Lym%	7.1 ↓	20.0-40.0	%
3 Gran%	90.7 ↑	50.0-70.0	%
4 Mid%	2.2 ↓	3.0-9.0	%
5 Lym#	1.21	0.80-4.00	10 ⁹ /L
6 Gran#	15.45 ↑	2.00-7.00	10 ⁹ /L
7 Mid#	0.37	0.10-0.90	10 ⁹ /L
8 RBC	4.44	3.50-5.00	10 ¹² /L
9 HGB	13.1	11.0-15.0	g/dL
10 HCT	38.7	37.0-48.0	%
11 MCV	87.2	82.0-95.0	fL
12 MCH	29.4	27.0-31.0	pg
13 MCHC	33.7	32.0-36.0	g/dL
14 RDW-CV	12.8	11.5-14.5	%
15 RDW-SD	45.2	35.0-56.0	fL
16 PLT	286	150-450	10 ⁹ /L
17 MPV	8.3	7.0-11.0	fL
18 PDW	15.8	15.0-17.0	%
19 PCT	0.238	0.108-0.282	%
20 P-LCR	15.8	11.0-45.0	%
21 P-LCC	45	30-90	10 ⁹ /L

WBC Message
Granulocytosis

RBC Message

PLT Message



Submitter:

Draw Time: 25/07/2025 14:40

Report Time: 25/07/2025 14:48

*The Report is responsible for this sample only. If you have any questions, please contact us in 24 hours.

Operator: admin

Received Time: 25/07/2025 14:40

Remarks:

Approver:

Validated Time:

Course in Hospital:

Patient is a case of acute pancreatitis, presented with complaints of pain abdomen. Relevant investigations were done. ESR - 39, TC - 15.35, Vit D3 - 23.1 other blood parameters within normal limits. USG abdomen done which suggestive of minimal ascites. She was treated conservatively with IV fluids, IV antibiotics, Inj. Arachitol and other supportive measures. Hemodynamically stable. Now she is being discharged with following advice,

Medication during Hospitalization:

IV FLUIDS

Inj. Pantocid 40mg IV 1-0-1

Inj. Arachitol IM stat

Inj. Paracetamol 1gm IV 1-1-1

Tab. Pankreoflat 25K 1-1-1

Condition on Discharge from Hospital:

Stable

Medication on Discharge:

CAP. COD LIVER OIL 1-0-1 (BEFORE FOOD)

CAP. DAROLAC 1-0-1 (AFTER FOOD)

TAB. PANKREOFLAT HD 1-1-1 (AFTER FOOD)

TAB. ANTOSID 1-0-1 (AFTER FOOD)

CAP. NEUROBION FORTE 1-0-1 (AFTER FOOD)

Follow-up Appointment:

Review SOS in General Surgery OPD

Advice on Discharge / Special Needs:

Nutritional Advise: Low fat + soft diet

Obtain Urgent Care in case of:

Pain abdomen, weakness.

Name of the Doctor : Dr. Shiva Shankar Bhat
Department : Laparoscopic Surgery
Doctor's Sign : 

Prepared By CHAITHRA K
Department Discharge Summary Unit
Date & Time 28/07/2025 @ 11:23AM

Instructions :

In Case of Emergency, Call Casualty Medical Officer at: 0824 - 2225533 or +91 7676987172 and for Ambulance Services, call - 0824 2225555

In case, you have given biopsy sample, collect it within one week in person from Lab/OPD or consult your doctor.

In case of OPD doctor consultation appointment you can contact 0824 6613282 / 0824 6613333/7846802333

For radiology service please contact 0824 6613192/8 at least one day in advance between 8.30am to 5.00pm

Please bring the copy of discharge summary whenever you visit the hospital on follow up.

Other Services:

1) For AJ Home Health Services (Case of Post-operative services, Geriatric Services) Contact: +917795473777 (09:00AM- 05:00PM)

2) For Health Check-up/Hospital cards Contact: 08246613165



A.J. Hospital & Research Centre

A unit of Laxmi Memorial Education Trust(R)

Kuntikana, Mangaluru - 575 004

Tel : +91 824 222 5533 / 222 9333 / 661 3333

Website : www.ajhospital.in

Mail : ajhospitalmangaluru@gmail.com



Discharge Summary

Name	: Ms. PRANATI RAMAKRISHNA BHAT	UHID	: MR550803
Age/Sex	: 13 Years / Female	IP No	: IP0000205134
Address	: 50, Hostota, Belakanda, Yellapur, , Uttara Kannada, Karnataka, India. - 581359	Adm.dt	: 26-07-2025 12:09 PM
Treating Consultant	: Dr. Shiva Shankar Bhat	Dis.Dt & Time	: 28-07-2025
Ref Dr / Hospital	: -	Ward	: 6D

Consultant Name:

Dr. Shiva Shankar Bhat (Laparoscopic Surgery)

Final Diagnosis:

- ACUTE PANCREATITIS

Allergies:

No known allergies

Chief Complaints:

Patient is a case of acute pancreatitis, presented with complaints of pain abdomen since 1 day .
Now admitted here for further management.

General Examination/Vital Signs on Admission:

Temperature: 98.6 degree F
Pulse Rate: 64/min
Respiratory Rate: 22/min
Blood Pressure: 100/70mmHg
Weight - 62kgs

Systemic Examination:

CVS: S1, S2 +, no murmur
RS: NVBS +, no added sounds
PA: Soft, non tender
CNS: Conscious, oriented

Lab investigations:

Reports are attached with file and handed over to attendants

Radiology Investigations:

26-07-2025

Ultrasound - Abdomen :

IMPRESSION: MINIMAL ASCITES.